

Medical Necessity and Coverage Determination Guidelines 2026

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1. Medical Necessity Definition

Medical necessity is defined as health care services or supplies needed to diagnose or treat an illness, injury, condition, disease, or its symptoms, and that meet accepted standards of medicine. Services must be: (1) ordered by a licensed healthcare provider, (2) necessary and appropriate for the diagnosis or treatment, (3) consistent with national clinical guidelines, and (4) not primarily for the convenience of the patient or provider.

2. Coverage Determination Process

Coverage determinations are made based on the member's benefit plan, clinical criteria, and evidence-based medical guidelines. The plan uses criteria from nationally recognized sources including InterQual, Milliman Care Guidelines, and specialty society guidelines. Clinical reviewers are licensed healthcare professionals with relevant clinical expertise. Initial determinations are made within 30 days for standard requests and 72 hours for urgent requests.

3. Preventive Care Services

Preventive care services are covered at 100% with no cost-sharing when delivered by in-network providers. Covered preventive services include USPSTF Grade A and B recommendations, ACIP immunization schedules, and preventive care guidelines for children and adolescents (Bright Futures). CPT codes for preventive visits: 99381-99387 (new patient) and 99391-99397 (established patient).

4. Chronic Disease Management

Members with chronic conditions including diabetes, hypertension, heart failure, COPD, and asthma are enrolled in disease management programs. Disease management services are covered including nurse care coordination, diabetes self-management training (DSMT, CPT 98960-98962), and cardiac rehabilitation (CPT 93798). HbA1c testing for diabetic members is covered quarterly without prior authorization.

5. Experimental and Investigational Services

Services classified as experimental or investigational are excluded from coverage unless the member is enrolled in an approved clinical trial. Clinical trial coverage includes routine costs associated with participation in Phase II-IV cancer clinical trials. Coverage is provided for trials sponsored by federal agencies or conducted under an IND application. Investigational medical devices may be covered under the Coverage with Evidence Development (CED) program.

6. Second Opinion Coverage

Members have the right to obtain a second medical opinion from any in-network specialist without referral. Second opinions for surgical procedures are strongly encouraged. Out-of-network second opinions are covered at in-network rates when in-network specialists are not available in the service area within a reasonable distance. Documentation of the second opinion may be required for certain high-cost procedures.

7. Coverage for Pediatric Services

Pediatric preventive care is covered as described in the Bright Futures guidelines through age 21. Developmental screenings, autism spectrum disorder screenings, and behavioral health assessments are covered without cost-sharing. Pediatric dental, vision, and hearing benefits are included as essential health benefits for members under age 19 in qualified health plans.

8. Mental Health Parity

Mental health and substance use disorder (MH/SUD) benefits must be provided on the same basis as medical and surgical benefits. Parity applies to: financial requirements, quantitative treatment limits, and non-quantitative treatment limits. Prior authorization criteria for MH/SUD services must be comparable to medical/surgical criteria. Annual parity analyses are conducted and available upon request.